SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Weekly paclitaxel + trastuzumab → trastuzumab continuation

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adjuvant · HER2-positive early breast cancer · NCCP 00815 · version 3 · 21-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
Paclitaxel chemotherapy and HER2-directed trastuzumab reduce recurrence risk after surgery in the selected early-breast setting. Trastuzumab then continues without paclitaxel.

**Schedule**
Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate.

## 1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

### Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Numbness / tingling, sometimes lasting
- ☐ Hair loss, muscle or joint pain
- ☐ Infusion reaction
- ☐ Mouth, bowel, skin and nail effects

### Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Infusion or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss the active phase, supportive medicines, fertility / menopause, treatment alternatives, monitoring, urgent contacts and when the plan will be reviewed.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00815 v3.

SACTCheck v0.83.8
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